

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

February 24, 2026

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

CAPT (b)(6),(b)(7)(C),(b)(7)(F) MD, MPH, CCHP, FAAFP
Chief Medical Officer
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Chief of Staff
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CDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Chief, Investigations Unit
ICE Health Service Corps

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FROM:

CDR (b)(6),(b)(7)(C),(b)(7)(F) PA-C
Investigator
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings
Oscar Duarte RASCON, (b)(6),(b)(7)(C)

On September 8, 2025, the U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of alien Oscar Duarte RASCON, (b)(6),(b)(7)(C). ICE detained Mr. RASCON, a 59-year-old Mexican male, at Eloy Detention Center (EDC), in Eloy, Arizona (AZ), from January 7 to 15, 2025, prior to his transfer and admission to a local hospital, and his subsequent death on September 8, 2025.

Prior to his detention, on September 10, 1976, Mr. RASCON entered the United States at or near Douglas, AZ, without admission or parole. Between June 6, 1984, and July 19, 2005, four Arizona law enforcement agencies charged Mr. RASCON with various crimes, for which Mr. RASCON served 21 years of incarceration.

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On January 6, 2025, ICE Enforcement and Removal Operations (ERO) Phoenix encountered Mr. RASCON at Arizona Department of Corrections (ADOC), in Phoenix, AZ, and determined he was unlawfully present in the United States. On the same date, ERO Phoenix issued Mr. RASCON Form I-862, charging him with violating Section 212(a)(6)(A)(i) of the Immigration and Nationality Act, as an alien having re-entered the United States without being admitted or paroled, and transferred Mr. RASCON to Florence Staging Facility (FSF), in Florence, AZ.

On January 6, 2025, FSF medical staff completed Mr. RASCON's prescreen and documented his incoherent speech, inability to walk and provide consent, (b)(6),(b)(7)(C) and his need for full assistance with activities of daily living (ADLs). Due to FSF's inability to provide safe, effective care, ICE transported Mr. RASCON to the Honor Health Florence Medical Center (HHFMC) emergency room (ER), with a transfer plan to EDC upon hospital discharge.

On January 7, 2025, EDC medical staff completed Mr. RASCON's intake, documented normal vital signs, unchanged status, and continued his treatment plan from ADOC. On the same day, an advanced practice provider (APP) completed Mr. RASCON's comprehensive health assessment, documented normal vital signs, noted an incomplete exam due to Mr. RASCON's agitation, admitted him to the medical housing Unit (MHU), and ordered hourly wellness checks and extensive ADL support.

On January 11, 2025, medical staff witnessed Mr. RASCON seize, called a medical emergency, and requested an emergency medical services (EMS) transfer to Banner Casa Grande Medical Center (BCMC) for further evaluation. On the same day, an APP evaluated Mr. RASCON upon his return and documented a verbal report from a BCMC staff member of Mr. RASCON's normal vital signs, negative head scan result, and no medication changes.

On January 15, 2025, EDC medical staff evaluated Mr. RASCON, documented his unchanged status, and transferred him to Phoenix Specialty Hospital (PSH) for a higher level of care.

Between January 16 and September 5, 2025, EDC RNs documented daily reports from PSH staff of Mr. RASCON's decline due to (b)(6),(b)(7)(C) seizures, and a clostridium difficile (bacterial) infection. On September 5, 2025, an EDC RN documented PSH staff reported Mr. RASCON's transfer to Banner Desert Hospital (BDH) due to multi-organ failure, a bladder infection, and a lung infection.

Between September 6 and 7, 2025, an EDC RN documented a BDH medical doctor (MD) reported Mr. RASCON's medical decline with a plan to intubate with bilevel positive airway pressure (BIPAP) (a medical procedure where a tube is inserted into the trachea to maintain ventilation). BDH MDs contacted Mr. RASCON's family, who declined to make decisions about his care, and the MD changed Mr. RASCON's code status to do not resuscitate (DNR) and do not intubate (DNI).

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On September 8, 2025, an EDC RN documented BDH staff reported Mr. RASCON's pending transfer to comfort/hospice care. On the same day, at approximately 2:43 p.m., a BDH MD declared Mr. RASCON deceased.

Mortality Finding:

As a result of Mr. RASCON's death, IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues requiring further study.

On December 11, 2025, Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) investigator, IHSC Investigations Unit, discussed the events surrounding Mr. RASCON's death with IHSC Chief Medical Officer Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) medical doctor, physician reviewer. The reviewers determined EDC provided Mr. RASCON medical care within the safe limits of practice and in accordance with applicable standards, and IHSC and EDC policies and procedures.

The IHSC Investigations Unit deferred an on-site mortality review and mortality review committee (MRC) meeting given Mr. RASCON's critical health status and serious evolving medical condition upon release from ADOC.

Therefore, the following summary of events was based on 1) Mr. RASCON's FSF, EDC, BCMC, PSH, and BDH medical records; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. RASCON'S EDC detention file; 5) applicable EDC policies and procedures; and 6) applicable ICE Detention Standards.

Summary of Events

On January 6, 2025, an FSF RN completed Mr. RASCON's prescreen and documented his inability to walk, provide consent, or form coherent speech; (b)(6),(b)(7)(C)
(b)(6),(b)(7)(C) with a suprapubic catheter in place (a catheter inserted into the bladder used to drain urine from the bladder into an exterior drainage bag); recent hospitalization for septic shock (widespread infection causing organ failure) and urinary tract infection (UTI); and his need for full assistance with ADLs, including feeding, toileting, grooming, dressing, ambulation, and bathing. Due to FSF's inability to provide safe, effective care, EMS transported Mr. RASCON to HHFMC's ER, with a transfer plan to EDC upon hospital discharge.

On January 7, 2025, an EDC RN completed Mr. RASCON's intake screen upon discharge from HHFMC and documented normal vital signs, unchanged status, and a continuation of his current

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plan from ADOC, to include: omeprazole (used to treat and prevent conditions caused by excessive stomach acid) 20 milligrams (mg), one tablet, orally, at night; hydroxyzine (antihistamine used to treat anxiety) 25 mg, two tablets, orally, three times daily when needed; tamsulosin (used to treat symptoms of an enlarged prostate) 0.4 mg, one tablet, orally, at night; divalproex delayed release (DR) (used to treat seizures) 250 mg, one tablet, orally, at night; divalproex DR 125 mg, one tablet, orally, every morning; weekly suprapubic catheter care; and monthly catheter change. On the same day, an APP completed Mr. RASCON's comprehensive health assessment, documented normal vital signs, noted an incomplete exam due to Mr. RASCON's agitation, admitted him to the MHU, and ordered hourly wellness checks, soiled diaper change at least every two hours, urostomy bag change every four hours, vital sign checks every eight hours, extensive ADL support, and a fall mat whenever in bed.

On January 11, 2025, an RN witnessed Mr. RASCON's seizure for approximately one minute while he was in bed. Medical staff transferred Mr. RASCON to the floor, called a medical emergency, requested EMS, and prepared to initiate cardiopulmonary resuscitation (CPR). Once transferred to floor, Mr. RASCON gained color, his breathing improved, and his pulse became palpable. Medical staff removed his clothes, placed automated external defibrillator (AED) pads on his chest out of abundance of caution (no shock delivered), and applied an oxygen mask, with a flow rate of 2 liters per minute, prior to EMS arrival for transport to BCMC's ER for further evaluation. On the same day, an EDC APP evaluated Mr. RASCON upon his return from the BCMC ER, and documented no discharge documents and receipt of a verbal report from a BCMC staff member of Mr. RASCON's normal vital signs, his negative head computed tomography (CT) scan (uses a series of x-rays and computer processing to create images of internal organs) result, and no medication changes.

Between January 12 and 15, 2025, EDC medical staff evaluated Mr. RASCON, documented his unchanged status, and transferred him to PSH, via van, for a higher level of care.

Between January 16 and February 22, 2025, EDC RNs documented daily reports from PSH staff that Mr. RASCON was stable, his overall condition remained unchanged, and they had no plan for discharge. During this time, PSH physicians started enoxaparin (a medication used to prevent blood clots), famotidine (antacid), aspirin, and cefepime (antibiotic used to treat UTIs), increased the frequency of hydroxyzine, and ordered physical therapy.

Between February 23 and June 2, 2025, EDC RNs documented daily reports from PSH staff that Mr. RASCON was declining due to Alzheimer disease processes (e.g., agitation, loss of communication, difficulty swallowing, lack of awareness, aspiration pneumonia, loss of movement, pressure sores), and he experienced two witnessed seizures. PSH physicians diagnosed Mr. RASCON with fever of unknown origin, treated him with intravenous antibiotics, increased his hydroxyzine frequency, and initiated orders for lorazepam and haloperidol for agitation. The physicians additionally prescribed pain relief medications, melatonin, metoprolol (a heart rate and blood pressure medication often used to provide relief), oxygen for comfort, and

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heparin (medication that prevents blood clots). Following a positive chest x-ray result for mild pneumonia, the physicians ordered intravenous antibiotics and a bedside swallow test (test to identify risk for choking), which resulted in nasogastric tube (a feeding tube that runs internally from the nose to stomach) placement due to aspiration risk. Mr. RASCON developed and received care for right and left foot pressure sores.

Between June 3 and September 4, 2025, EDC RNs documented daily reports from PSH staff that Mr. RASCON was stable. He experienced one witnessed seizure, and his overall condition and treatment plan remained unchanged, except for an order for palliative care and the addition of three antibiotics for recurring pneumonia due to aspiration. The physicians ordered additional pain relief medication, glycopyrrolate (controls stomach acid production), and intravenous iron, iron sucrose, and erythropoietin (stimulates red blood cell production) to treat iron deficiency anemia. The physicians diagnosed him with a clostridium difficile (bacterial) infection, initiated contact precautions, and treated him with intravenous vancomycin (antibiotic). Mr. RASCON experienced bilateral feet swelling, and the PSH staff continued to provide wound care for sacral (bony area at the top of the buttocks), right hip, and bilateral elbow wounds.

On September 5, 2025, an EDC RN documented PSH staff reported Mr. RASCON's transfer to BDH due to multi-organ failure, a bladder infection, and a lung infection.

Between September 6 and 7, 2025, an EDC RN documented BDH staff reported Mr. RASCON's diagnoses of septic shock, hypoxia (an absence of enough oxygen to sustain bodily functions), renal disease, UTI, and pneumonia. BDH physicians ordered intubation with BIPAP, norepinephrine (a potent vasoconstrictor used to treat low blood pressure), and propofol (intravenous medication used to induce sedation). BDH physicians contacted Mr. RASCON's family, and his family chose not to make decisions over his care. The BDH MD changed Mr. RASCON's code status to DNR and DNI.

On September 8, 2025, an EDC RN documented BDH staff reported Mr. RASCON's pending transfer to comfort/hospice care.

- At approximately 2:43 p.m., a BDH MD pronounced Mr. RASCON dead.

The final death certificate revealed Mr. RASCON's cause of death was complications of (b)(6),(b)(7)(C). The medical examiner determined the manner of death as natural.

The reviewers used the following accreditation standards for this review: ICE Performance-Based National Detention Standards (PBNDS) 2011, with 2016 revisions; and National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The reviewers identified the following program strengths and recommendations during this review.

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Strengths

The MRC identified the following strengths: 1) EDC medical staff completed a timely intake for Mr. RASCON. 2) EDC medical staff completed a timely comprehensive examination of Mr. RASCON. 3) EDC medical staff provided timely response to Mr. RASCON's chronic medical conditions. 4) EDC staff responded to the health-related emergency within four minutes. 5) EDC medical staff transferred Mr. RASCON in a timely manner to an appropriate facility for emergency care and further evaluation.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Systems Support Division (HSSD), DAD of Clinical Services Division (CSD), and DAD of Health Care Compliance Division (HCCD).
- The IHSC DAD of HSSD will share these findings with EDC's health services administrator (HSA) for review, dissemination to ICE and EDC's facility administrator, and any action(s) deemed appropriate.
- The IHSC DADs of CSD and HCCD will share these findings with the respective IHSC CSD and HCCD units for review and any action(s) deemed appropriate.
- EDC's HSA and the respective IHSC CSD and HCCD units will ensure a corrective action plan, if applicable, is implemented and sustained.

End of Report.